

Pharmacovigilance Individual Case Processing and Narrative Writing
A Simulated Study Project

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Purpose: Prepared for academic and pharmacovigilance training purposes using simulated data only.

Data Catalogue – Column Descriptions

1. Case ID

A unique alphanumeric identifier assigned to each Individual Case Safety Report (ICSR) to ensure traceability and case-level tracking throughout processing.

2. Reporter Type

Indicates the source of the safety report, such as Healthcare Professional (HCP) or Consumer, used to assess report credibility and regulatory relevance.

3. Patient Info

Contains basic patient demographic information (e.g., age and gender) sufficient to identify a unique patient without revealing personal identifiers.

4. Suspect Drug

The medicinal product suspected to be associated with the reported adverse event, as identified by the reporter.

5. Indication

The medical condition or reason for which the suspect drug was prescribed or used.

6. Adverse Event

The untoward medical occurrence reported in association with the use of the suspect drug, regardless of causal relationship.

7. MedDRA PT

The Medical Dictionary for Regulatory Activities (MedDRA) Preferred Term used to standardise the reported adverse event for regulatory consistency.

8. System Organ Class (SOC)

The MedDRA System Organ Class corresponding to the coded Preferred Term, representing the physiological system affected.

9. Seriousness

Indicates whether the adverse event is classified as serious or non-serious based on regulatory seriousness definitions.

10. Seriousness Criteria

Specifies the regulatory criterion that qualifies the case as serious (e.g., hospitalisation, life-threatening, death), if applicable.

11. Country

The country from which the safety case was reported, relevant for regulatory reporting requirements and regional compliance.

12. Dose

The strength and frequency at which the suspect drug was administered, as reported in the case.

13. Duration of Therapy

The length of time the patient was exposed to the suspect drug prior to onset of the adverse event.

14. Concomitant Medications

Other medications taken by the patient at the time of the adverse event, which may contribute to or confound the safety assessment.

15. Relevant Medical History

Clinically relevant past or ongoing medical conditions that may influence interpretation of the adverse event.

16. Action Taken with Suspect Drug

Describes any action taken regarding the suspect drug following the adverse event (e.g., withdrawn, dose reduced, not changed).

17. Outcome

The clinical status of the adverse event at the time of reporting (e.g., recovered, recovering, not recovered, unknown).

18. Event Onset Date

The date on which the adverse event first occurred or was first noticed by the patient or reporter.

19. Case Receipt Date

The date on which the safety report was first received by the organisation, used to calculate regulatory timelines.

20. Regulatory Due Date

The calculated deadline by which the case should be reported to regulatory authorities, based on seriousness and receipt date.

21. Note (RDD)

A clarification or justification note related to the Regulatory Due Date, including delays, assumptions, or training-related remarks.

22. Submission Status

Indicates whether the case has been submitted, not submitted, or pending submission (for training simulation purposes only).

23. Compliance Risk

An assessment of potential compliance concern (e.g., low, medium, high) based on data completeness and timeline adherence.

24. Case Validity (Yes/No)

Indicates whether the case meets the four minimum criteria for a valid ICSR: identifiable patient, identifiable reporter, suspect drug, and adverse event.

25. Reason if Invalid

Documents the specific missing element(s) that resulted in the case being classified as invalid.

26. Drug–Event Plausibility

An assessment of whether the reported adverse event is medically reasonable for the suspect drug based on known safety profiles.

27. Plausibility Justification

A brief rationale explaining the plausibility assessment, referencing known pharmacology or recognised adverse reactions.

28. QC Status

Indicates whether the case has passed or failed basic quality control checks related to completeness, consistency, and documentation.

29. Final Compliance Status

A consolidated status reflecting overall case readiness from a documentation and compliance perspective (e.g., compliant, non-compliant, requires follow-ups).